Shock – Section Summary

Septic • Obstructive • Cardiogenic • Haemorrhagic • Tamponade

Critical Care Board Review • Section 5 • MCQ-Oriented

1. Septic

- Persistent hypotension → reassess, US → **norepinephrine**
- Daily antibiotic review → **de-escalation**
- Source control: drain, debride, remove devices
- Lactate not clearing → consider other causes

2. Obstructive

- Massive PE + shock → check contraindications → **thrombolysis**
- Tamponade: **equalisation of diastolic pressures**; suspect → echo
- Tension pneumothorax on ventilator → **needle decompression**
- Chest tube air leak post-op → observe

3. Cardiogenic

- Large MI at non-PCI centre → **transfer for PCI**
- New murmur + shock → **VSR**
- RV infarct/hypotension → phenylephrine/fluids

4. Haemorrhagic

- Loss > 40%: diastolic dysfunction, ↓ filling, ↓ SV

5. High-Yield MCQ Pearls

- ☑ Tamponade = diastolic equalisation
- ☑ PE + shock → lysis

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